

Recruitment and Eligibility of Participants

3.1 Overview

Both Cases and Controls are identified, recruited, and screened for eligibility in order to participate in the study. Interviewers have been trained in techniques to use for explaining the study to potential participants and gaining their cooperation. Refer to the Interviewer Manual for additional information.

3.2 Study Eligibility Criteria

Eligibility is restricted to women who are between the ages of 18 and 74 years old (inclusive) and have lived within one of several census-defined study districts for at least one year (shown in table below).

Region	Study Districts	
Greater Accra	Accra Metropolitan Tema Adenta Ga East Ga West	Weija (Ga South) Ashaiman Ledzokuku/Krowor Dangbe East Dangbe West
Central	Awutu Senya	
Eastern	Suhum/Kraboia Coaltar Akwapem South	
Ashanti	Kumasi Metropolitan Atwima Kwanwoma Atwima Nwabiagya Afigya Kwabre Mampong Municipal	Bosumtwi Ejisu Juaben Kwabre East Bekwai Municipal

Among Cases, eligibility is further restricted to women who were either diagnosed with breast cancer within the past year or were referred to one of the study hospital for care related to breast cancer that was diagnosed in the past year. Eligible Cases include women diagnosed with invasive and/or *in situ* breast cancers; women diagnosed only with benign breast conditions, such as cysts or

fibroadenomas, are not eligible Cases. Among Controls, eligibility is restricted to women who have never been diagnosed with breast cancer. All potential participants (both Cases and Controls) will be asked a series of questions to determine their eligibility.

3.3 Case Recruitment

All potential Cases should be approached for study participation. A potential Case is a woman who meets any of the following criteria:

- Has been referred for a breast tissue biopsy by a doctor, nurse, or clinician (including a traditional healer);
- Self-presents without a referral for a breast tissue biopsy and is suspected to have breast cancer;
- Has already had a breast tissue biopsy (either at KBTH, KATH, PLH, or another hospital), was diagnosed with invasive or *in situ* breast cancer in the past year, and is returning for non-surgical treatment; or
- Has already had a breast tissue biopsy (either at KBTH, KATH, PLH, or another hospital), was diagnosed with invasive or *in situ* breast cancer in the past year, and is returning for breast surgery

It is expected that the majority of potential Cases will be women who present to the hospital for a breast tissue biopsy. However, it is very important for this study that all potential Cases are sought out and approached to be in the study. Potential Cases approached to be in the study will be assigned a Participant Identifier (PID), even if they do not agree to participate or are ineligible (Refer to Chapter 2.2.3, Assigning a PID).

The Interviewer assigned to the potential Case will briefly explain the study to each potential Case and will provide her with a study brochure prior to assessing her eligibility to participate. If the woman expresses interest in learning more about participation, the Eligibility Screener Form (Appendix 3-1) should be administered by the Interviewer. Version 2.0 of the Eligibility Screener Form replaced Version 1.0 in January 2014. Procedures for administering the Eligibility Screener Form are described in Chapter 4 in the Interviewer Manual.

If the woman meets all of the eligibility criteria as determined by the Eligibility Screener Form, each component of study participation and the Informed Consent should be explained. The Interviewer

should provide the participant with a signed copy of her Informed Consent document (Refer to Chapter 4 of this Manual).

3.4 Control Recruitment

Population Controls will be selected through a process of (i) enumerating all women within a defined geographic area known as an Enumeration Area (EA), and (ii) inviting a sample of these women to participate in the study, based on specific age and residence criteria.

3.4.1 Population Enumeration

Enumeration will be carried out by Professor Richard Biritwum of the University of Ghana. The purpose of enumeration is to provide a defined population of known size, age, and residency from which to sample study Controls. Each study district can be sub-divided into enumeration areas. During enumeration, the location of each home or dwelling within an enumeration area is documented. For this study, one or more enumeration areas in each study district were chosen randomly, and all households within the selected area were enumerated. Household location, and the number, age, and sex of inhabitants was recorded using a standardized tracking form; when possible, name and telephone number were also be obtained. Once complete, the data was compiled electronically. A sample of women frequency-matched to the anticipated age (5-year groups) and district of residence distribution of the study Cases was selected and their names and contact information were provided to investigators at KBTH, KATH, and PLH. These lists served as the source for Control recruitment.

3.4.2 Control selection

Study Managers actively manage the recruitment of potential Controls because Controls should be enrolled in the study at approximately the same rate as Cases (i.e. Controls should not be recruited significantly later than or earlier than Cases).

Potential Controls should be contacted by telephone or by home visit, whichever is found to be more effective or a combination thereof, in order to explain the study and invite them to participate. If a selected Control cannot be reached by phone or is not at home, additional attempts must be made to contact her at different times of the day and using different methods, if both address and phone number are provided. Each contact attempt and the result should be documented using Record of Calls. At least five contact attempts must be made before determining that the potential Control cannot be located and replacing the woman with another potential Control.

When contact is made, the study should be explained briefly to each woman prior to assessing her eligibility to participate. The Interviewer should use the techniques covered during training to gain cooperation from the potential participant. The Interviewer administers the Eligibility Screener to confirm eligibility. If the woman meets all of the eligibility criteria as determined by the Eligibility Screener form, each component of study participation is then explained to her and an appointment made for her to come to the hospital. In the Ashanti region, potential Controls should be informed that the study can provide transportation to and from the hospital, if necessary. In other regions, potential Controls should be informed that the study can provide assistance with transportation expenses, if necessary. On the day of the study appointment, informed consent is obtained and the Interviewer should provide the woman with a copy of her signed Informed Consent document. Then, the Risk Factor Questionnaire, anthropometry, and biological specimens components of the visit should be completed.

If an eligible Control is willing to participate but multiple unsuccessful attempts have been made to schedule the hospital visit, as a last resort the study can be completed in the field. It is strongly preferred that the study be completed in the hospital to ensure privacy for the interview, complete specimen collection, and complete anthropometry. However, it is also important to maximize participation rates and this may sometimes necessitate completion in the field.